

Switch ART

Aim to switch all patients to dolutegravir (DTG). Resistance to DTG is rare and DTG provides rapid VL suppression and has minimal side effects.

Check if patient eligible to switch, or restart, DTG-based regimen same day:

Does patient's ART regimen include LPVr or ATVr?

No

- Patient on one of following:
 - TDF + FTC + EFV (TEE)
 - ABC + 3TC + EFV
 - AZT + 3TC + EFV
 - AZT + 3TC + DTG
 - NVP-based regimen
- If patient on regimen other than above, discuss with doctor, HIV expert or HIV hotline ↗ 178.

Switch to DTG-regimen today, regardless of VL.

- Check VL result done in last 12 months:
 - If VL ≥ 50: continue to switch but assess and support adherence ↗ 173.
 - If VL not done in last 12 months, do it at this visit. No need to wait for results before switching.

- Switch to **TDF + 3TC + DTG** same day. Fixed combination dose tablet known as **TLD** Record as **TLD1** if patient never failed a previous ART regimen and **TLD2** if patient failed a previous ART regimen.
- If known kidney disease (eGFR ≤ 50 or creatinine > 85 in pregnant woman), use instead **ABC + 3TC + DTG**. Fixed combination dose tablet known as **ALD**. Record as **ALD1** if patient never failed a previous ART regimen and **ALD2** if patient failed a previous ART regimen.
- Only if previous hypersensitivity reaction to ABC, use instead **AZT + 3TC + DTG**.
- If unsure, discuss with experienced ART doctor, HIV expert or HIV hotline ↗ 178.

- If VL done in last 12 months < 50, continue routine viral load monitoring ↗ 112.
- If VL done in last 12 months ≥ 50, repeat VL in 3 months (or 4-6 weeks if pregnant).

- Ask about other medications: especially TB or epilepsy treatment, contraceptives and calcium, iron, zinc, antacids, metformin. Check if ART needing adjusting ↗ 118.
- Check if bloods needed ↗ 112: if starting TDF, take baseline creatinine. If stopping TDF, check HBsAg before switching ART. If starting AZT, check Hb.
- Dose ART correctly according to chart ↗ 116.

Yes

Has patient been on LPVr or ATVr for 2 or more years?

No

Manage further according to last two VL results, taken at least 2 years after starting LPVr or ATVr (if VL unknown, discuss):

Both VL results < 1000

VL < 1000

- Continue same regimen and assess and support adherence ↗ 173.
- Repeat VL in 3 months:

VL ≥ 1000

Both VL results ≥ 1000

Virological failure confirmed

- If pregnant, discuss with HIV expert/hotline ↗ 178.
- Assess adherence in last 6-12 months by checking script for pharmacy refills and notes for clinic appointment attendance¹.
- Have refills been collected > 80%² of time or has patient attended > 80%³ clinic visits?

No

- Adherence considered poor. Resistance test is *not* indicated.
- Switch to **TDF + 3TC + DTG** same day. This is available in a fixed combination dose tablet called **TLD2**.
 - If known kidney disease (eGFR ≤ 50 or creatinine > 85 in pregnant woman), use instead **ABC + 3TC + DTG**. Fixed combination dose tablet known as **ALD2**.
 - Only if previous hypersensitivity reaction to ABC, use instead **AZT + 3TC + DTG** (2nd-line).
- If unsure, discuss with ART doctor, HIV expert or HIV hotline ↗ 178.
- If restarting ART, manage further → 116.
- Repeat VL in 3 months.

Yes

- Adherence considered good.
- Discuss need for resistance testing and choice of new ART regimen with HIV expert, infectious disease specialist, third line ART committee or HIV hotline ↗ 178.

ABC – abacavir; **ATVr** – atazanavir/ritonavir; **AZT** – zidovudine; **DTG** – dolutegravir; **EFV** – efavirenz; **FTC** – emtricitabine; **NVP** – nevirapine; **LPVr** – lopinavir/ritonavir; **TDF** – tenofovir; **VL** – viral load; **3TC** – lamivudine

¹If available, also do drug level on urine or blood specimen: adherence is considered good if medications are detected in patient's urine/blood. ²Calculate adherence % for pharmacy refills: 'number of *actual refills done* during period assessed' ÷ 'number of months in period assessed'. Then x by 100. ³Calculate adherence % for clinic attendance: 'number of scheduled visits *actually attended* by patient during period assessed' ÷ 'number of scheduled visits during period assessed'. Then x by 100.